

OPTIMIZERX CORP OPRX

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by

templargin

			2026	2027
Price:	4.68	EPS	0	0
Shares Out. (in M):	22	P/E	0	0
Market Cap (in \$M):	102	P/FCF	0	0
Net Debt (in \$M):	3	EBIT	0	0
TEV (in \$M):	106	TEV/EBIT	0	0

Description

OPRX is a microcap and is best suited for small funds and PAs.

Summary

OPRX was written up by rhianik in Feb 2025 and today trades 30% lower despite having become a much stronger company.

OptimizeRx is the rare thing in microcap land — a digital-health company that is actually profitable — and the market is treating it like it's going out of business. The stock did a full round-trip over the last sixteen months: ~\$6 in February 2025, up to ~\$22 by late 2025, and back down to \$4.68 today, a fresh 52-week low printed this very week. It now trades at roughly 1x sales, 5x EBITDA, and a 20% free-cash-flow yield, with net debt of about \$3 million against \$20m+ of EBITDA and ~\$19m of free cash flow generated last year.

In 2025 OptimizeRx grew revenue 19% to \$109m, doubled adjusted EBITDA to \$24.3m, turned its first clean GAAP profit (\$5.1m), threw off \$19m of free cash flow, and crossed the "Rule of 40" line for the first time. Then in early 2026 it refinanced its expensive debt down by 600+ basis points. The fundamentals got better and the multiple got cut in half.

The reason the multiple got cut: 2026 is a down year. Management cut full-year revenue guidance twice in six months — from \$118–124m last November to \$95–100m in May — as pharma customers paused spending into the "Most-Favored-Nation" (MFN) drug-pricing fight, shortened their contracts, and one large client both pulled back and got mishandled. However, this is a profitable, cash-generative in a one-year air pocket, abandoned at trough multiples, with a new free call option.

Reasons to buy

It's cheap, it's profitable. At \$4.68 you pay 1x sales and 5x EBITDA for a 67%-gross-margin business that generated \$19m of free cash flow last year — a 20% FCF yield on the equity. Net debt is \$3.4m and shrinking; the company just bought itself room by refinancing 12.5% debt down to 6%. A profitable, net-cash company at an \$100m market cap with a scarce, decade-built distribution asset is hard to permanently impair. If the public market won't pay up, the private market will — this is exactly the kind of bolt-on (authenticated HCP reach, pharma relationships, patented DTC targeting) that a Veeva, Doximity, IQVIA or PE buyer could pay a premium for.

The operating-leverage model finally proved itself. For years OptimizeRx was a "trust me, the margins are coming" story. In 2025 they came. Revenue +19% drove EBITDA from \$8.8m to \$24.3m — gross margin expanded from 64.5% to 67.3%, and the largely fixed cost base did the rest. Yes, part of that was cost discipline (opex down, stock-comp down \$4.5m), and the bears are right that you can only cut once. But the structural point stands: this is an asset-light data-and-messaging business with near-zero capex where incremental revenue drops to the bottom line. Even on this year's lower revenue, management is holding EBITDA guidance at \$21–25m. That tells you the cost base is now sized to print cash through the trough (August will show whether this is real).

Free call option — the programmatic pivot. OptimizeRx spent a decade building a proprietary, authenticated, "bot-free" network into EHR and e-prescribing systems reaching 2m+ HCPs — and historically sold it the slow way, as managed-service campaigns. They're now opening that inventory to the demand-side platforms (DSPs) that buy ~80% of digital promotional dollars programmatically. In January they mapped their patented Micro-Neighborhood DTC audiences to Experian's identity graph; in May they launched the programmatic EHR connection; days later DeepIntent became the first DSP to plug in (live Q3 2026). Management thinks this channel could eventually be "comparable in size to our current HCP business" and says they're "the first ones to connect at scale." They're utilizing less than 10% of their network inventory today. None of this is in 2026 guidance, and revenue only starts flowing in Q4 2026 into the 2027 renewal cycle.

Why does this opportunity exist?

Two guidance cuts in six months torched the "new management sets beatable targets" premise. New CEO Silvestro set a 2026 guide in November and cut it within two months. That's a credibility event, and credibility events make growth and quality investors walk away regardless of valuation. But "the CEO lost the room on guidance" and "the equity is mispriced" can both be true at once.

It screens as a melting ice cube. Revenue is declining (–10% in Q1, FY2026 guided down ~11%). Growth screens reject it on the revenue line; quants reject it on the negative-revisions tape; value screens reject it because it's a \$90m microcap with goodwill at 63% of assets and a hairy story. It falls through every crack — too shrinking for growth investors, too small and ugly for value investors, too unloved for momentum.

The whole digital-health cohort is in the penalty box. Doximity is down ~67% off its highs, Definitive Health is down ~96% from its IPO, Phreesia cut its guide too. When the comp set is getting de-rated wholesale, even a stabilizing business gets thrown out with it. That's usually when the mispricing is largest.

MFN is a scary headline that the market is extrapolating to infinity. "Most-Favored-Nation drug pricing will crush pharma marketing budgets forever" is the bear narrative. More likely it's a one-to-two-year air pocket of caution that laps. Markets are bad at pricing the difference between permanent impairment and

temporary pause.

It's an illiquid microcap with guided revenue decline.

Layout

I'll start with how pharma actually spends its marketing money — the shift from sales reps to digital, why the e-prescribing screen became the most valuable real estate in the building, and a quick primer on programmatic advertising — because you can't underwrite OptimizeRx without it. Then we'll get into the company: history, the bubble and the round-trip, the business model and the all-important EHR network, customer and channel concentration, management, and the programmatic pivot in detail. I'll cover the numbers and my forward estimates, then valuation and risks.

Pharma marketing

Pharmaceutical companies spend something like \$30–35 billion a year in the US promoting their drugs, and for most of the last century almost all of it went to one thing: a person in a nice suit standing in a doctor's office. The practice is called "detailing," and it's old — the first recognizable drug reps were the "detail men" of the early 1900s who'd visit physicians to walk them through (to "detail") a product. By the 1990s and 2000s, Big Pharma ran armies of them — tens of thousands of reps, free lunches, branded pens, the whole circus. Detailing worked because medicine is a trust business and a prescription is a high-stakes, expert decision. You couldn't sell a drug with a billboard. You had to get in front of the doctor.

Two things broke the old model. First, access collapsed — hospital systems and large practices started locking reps out, and a majority of physicians today will not see a rep at all, or will give one a few rushed minutes a year. Second, COVID finished the job by making in-person detailing temporarily impossible and permanently less normal. So the budget had to go somewhere, and it went where the doctors actually are: on a screen, inside their workflow.

Which screen? Here's the key insight. A doctor spends their day inside an electronic health record (EHR) — the software they use to pull up a patient, review the chart, and, critically, write the prescription. The mass adoption of EHRs in the US was essentially mandated by the 2009 HITECH Act, which dangled tens of billions of dollars of incentives to digitize medical records. Within a few years, paper charts were gone and essentially every prescription in America started life as a few clicks inside an EHR or an e-prescribing (eRx) system. That moment — the doctor, looking at a specific patient, about to choose a drug — is the single most valuable instant in all of pharma marketing. It's the point of decision. It's the equivalent of a Google search ad showing up at the exact moment you've decided to buy, except the buyer is an MD and the purchase is a branded therapy.

OptimizeRx's whole existence is built on owning real estate at that moment. Over a decade it negotiated integrations into the EHR and eRx platforms — the plumbing — so that a pharma brand can place a message in front of the right prescriber inside the clinical workflow.

Three types:

- Brand/therapeutic messaging: a banner to the prescriber about a relevant therapy for the patient type in front of them.
- Financial messaging / affordability: showing the doctor (and pushing to the pharmacy) the copay card or savings offer for a branded drug, right at prescribe — which measurably lifts the odds the patient actually fills and stays on an expensive branded script instead of defaulting to a generic. This is genuinely valuable to pharma and the reason a chunk of OptimizeRx's revenue is stickier than the rest.
- Pharmacy/awareness alerts: telling the prescriber which pharmacy can actually fill or stock the drug.

The reason this is a business and not just an ad slot is the network. Getting integrated into EHR and eRx systems is slow, regulated, trust-laden plumbing work — you're touching clinical workflow and prescriber data, so it took OptimizeRx years and a web of channel-partner contracts to assemble reach to ~2m providers. That's the moat, such as it is: not the messaging tech, but the authenticated, hard-to-replicate distribution.

The other half of the story is the patient — direct-to-consumer (DTC). The US is one of only two countries on earth that lets drug companies advertise prescription drugs straight to consumers (hence every nightly-news ad with a smiling couple and forty seconds of side effects). DTC is a huge slice of pharma spend, and it's gone digital and "programmatic" like all advertising. OptimizeRx bought its way into this in 2023 with Medicx, which brought a clever, patent-protected approach called Micro-Neighborhood Targeting: instead of tracking individuals (a privacy minefield in healthcare), it uses de-identified claims data to score geographies — down to the 9-digit ZIP code, of which there are 35 million — for where eligible patients are likely to live, and then targets ads to those micro-neighborhoods across connected TV, social, audio, and the rest. Privacy-safe, and it dovetails with the HCP side: hit the doctor and the patient around the same therapy.

A last bit of plumbing you need, because it's the crux of the new catalyst: programmatic advertising. Most digital ads today aren't sold by salespeople; they're auctioned in milliseconds. Advertisers buy through demand-side platforms (DSPs) — the software that decides, in real time, which impression to bid on. Publishers expose their ad inventory through supply-side platforms (SSPs). Roughly 80–90% of digital display dollars flow through this programmatic pipework. For a decade, OptimizeRx sold its EHR inventory the artisanal way — bespoke managed-service campaigns, negotiated one at a time, lumpy and labor-intensive. The pivot is to expose that same scarce, authenticated EHR inventory as an SSP into the DSPs — to let the 80% of the market that buys programmatically reach the point of care the way they buy everything else. That's what the Experian, programmatic-launch, and DeepIntent announcements are. If it works, OptimizeRx stops being a boutique and becomes a toll road.

Company

History, the bubble, and the round-trip

OptimizeRx was founded in 2006 in Rochester, Michigan (it's now headquartered in Waltham, Massachusetts) and built the point-of-care messaging business described above. It bolted on capabilities over the years — CareSpeak (2018), RMDY (2019), EvinceMed (2022) — but the two events that matter for an

investor today are the 2021 bubble and the 2023 Medixx deal.

Like every digital-health name, OptimizeRx caught the 2020–2021 mania. The stock ran to roughly \$98 in late 2021 on a ~28x sales multiple — a genuinely absurd valuation for a sub-\$60m-revenue company — and management raised ~\$65m of equity at \$49.50 a share in February 2021. Then reality arrived: in 2022 management slashed revenue-growth guidance from ~35% to ~6%, blamed slow drug approvals and macro, and the stock lost ~86%. So far, a totally ordinary bubble-and-bust.

In October 2023 OptimizeRx made its big swing: it acquired Medixx Health for \$95m (\$83m cash, funded partly with a new \$40m term loan, plus ~\$12m of stock), buying its way into DTC and the Micro-Neighborhood targeting tech. The strategic logic — own both the HCP and the patient side of a drug launch in one omnichannel platform — is sound. The execution was not. The deal landed with poor timing, the integration was clumsy, the market was simultaneously shifting DTC from managed-service to self-service, and the combined company spent 2024 with a goodwill impairment (\$7.5m) and a GAAP loss (\$20m).

Which brings us to the round-trip that created today's setup. The stock bottomed around \$6 in early 2025, the Street fell in love again with the operating-leverage story (and an activist), and it ran back to ~\$22 by late 2025. Then 2026 happened: two guidance cuts, a –10% revenue quarter, the MFN scare, a mishandled large client — and the stock round-tripped right back down to \$4.68. Net of all the drama, the equity is roughly where it was at the early-2025 low, but the business in between went from loss-making to \$24m of EBITDA and \$19m of FCF. That gap — flat-to-lower stock, materially better fundamentals — is the trade.

Products

OptimizeRx sells pharma manufacturers (100+ of them) access to physicians and patients across an omnichannel network:

- The HCP point-of-care business — the core, the EHR/eRx messaging plumbing described above, including the stickier financial-messaging/affordability piece.
- The DTC business (ex-Medixx) — Micro-Neighborhood Targeting across programmatic channels.
- DAAP ("Dynamic Audience Activation Platform") — the AI/ML layer on top that decides which HCPs to target based on the patient care journey, and increasingly sold as a data subscription rather than a one-off campaign.

Revenue is mostly recognized over time as messages/audiences are delivered (transactional/programmatic), with a growing subscription layer (DAAP data). Gross margin runs in the mid-to-high 60s because the main cost of revenue is the revenue-share paid to the EHR/eRx channel partners. The strategic direction is to push more revenue into recurring subscriptions (DAAP data exited 2025 at a ~10% revenue run-rate, up from <5%, growing 45% in Q1 2026) to make the top line less lumpy and the margins higher.

The concentration problem

This is the single most important risk to internalize, so I'm putting it in the body rather than burying it in the risk list. OptimizeRx is concentrated on both sides of its P&L:

- Customers: the top 5 pharma clients are ~47% of revenue, and in 2025 three separate customers were each over 10% of revenue. The May guidance cut was driven substantially by one large client pulling back. Net revenue retention has eroded from 121% (2024) to 116% (2025) to 110% (Q1 2026) — still above 100%, but the trend is the wrong direction.
- Channel partners: even scarier, in 2025 a single channel partner accounted for 44% of revenue (up from 29% the year before). The network OptimizeRx resells is itself concentrated. If that partner renegotiates or walks, it's a different company.

Concentration cuts both ways — it's why the stock is cheap, and it's why the stock could stay cheap or get cheaper. A single large pharma brand re-engaging, or one big new DSP relationship, swings the model meaningfully in the other direction. But you have to size this honestly: this is not a diversified annuity.

Management

I haven't spoken to CEO but did speak to Ed and Andy multiple times. They seem confident about their current position in market and prospects of the business going forward. I haven't noticed any red flags with them.

Silvestro (a long-time insider — ex-CCO, then President) took over as permanent CEO in March 2025, replacing William Febbo, in a transition that the proxy makes clear was activist-influenced ("CEO search fees" and "shareholder activist related fees" were explicitly carved out of the bonus pool). CFO Ed Stelmakh is a steady hand. The board is genuinely upgraded — chair Lynn O'Connor Vos (ex-CEO Grey Healthcare), a former Walgreens CEO (Greg Wasson), and a Microsoft Health & Life Sciences COO added in April 2026. Six of seven directors are independent and the board is de-classified (annual elections). And the activist (Whetstone, 8%) is the kind of shareholder who keeps everyone honest.

The candor on the Q1 call is worth quoting, because it's unusually human for a public-company CEO. On the client that drove the cut: "Part of those are MFN related. Part of them, honestly, is just we did not execute well. In that account... we sort of know where we fumbled the ball a little bit there." I'll take a CEO who admits he fumbled over one who blames the weather. But the flip side is the credibility hit: this is the same team that set a 2026 number in November and cut it 12% by May, then asked the Street to trust the new number — "I do not expect that we will need to adjust again... we are giving the best view that we have got right now." They have to actually hit it.

The programmatic pivot

January 2026 — Experian collaboration: maps OptimizeRx's patented Micro-Neighborhood DTC audiences (ZIP+9 across 35m+ micro-neighborhoods) onto Experian's identity graph for privacy-safe identity resolution. Translation: it makes the DTC audiences portable and measurable across the big media channels,

closing the "we lost half the audience in onboarding" gap that plagues healthcare DTC.

May 11, 2026 — Programmatic EHR launch: a connection point that lets media buyers activate OptimizeRx's authenticated EHR point-of-care inventory inside their existing programmatic (DSP) workflows. This is the toll-road move — exposing scarce, "bot-free," authenticated EHR inventory to the ~80–90% of digital spend that buys programmatically, and to the \$19B+ pharma digital ad market.

May 21, 2026 — DeepIntent integration: DeepIntent becomes the first healthcare DSP to plug into that EHR network (live expected Q3 2026). First proof point.

Why this could be big: OptimizeRx says it's utilizing less than 10% of its network inventory today, because the managed-service sales motion is the bottleneck, not the supply. Connecting to DSPs removes the bottleneck and lets demand find the inventory at programmatic speed and scale. Management's framing — that this channel could become "comparable in size to our current HCP business over the long term," with revenue starting in Q4 2026 and "really flowing" into the 2027 renewal cycle — is obviously promotional, and none of it is in guidance. But the direction is right, the inventory is genuinely scarce, and being first to connect it at scale is worth something. Treat it as a real option with a wide range of outcomes, not a line in your model.

Valuation

SP	\$4.68
SO (basic)	18.76m
Options	2.37m
RSUs	0.72m
FD SO	21.85m
MC (FD)	\$102m
Cash	\$20.2m
Debt	\$23.6m
Net debt	\$3.4m
EV	\$106m

You're paying 1x sales and 5x EBITDA for a profitable, free-cash-generative business with normalized GM close to 70%.

Risks

Concentration, both ends. Top 5 customers ~47%; three clients each >10%; one channel partner at 44% of revenue. Any one of these moving against the company is a material event. This is the opposite of a diversified annuity.

Revenue might not be in an air pocket — it might be a melting ice cube. If the moat is genuinely eroding and pharma's pullback is structural rather than MFN-timing, then "stabilization" never comes, the multiple is correct. The line between "temporary pause" and "permanent impairment" is the entire debate.

MFN / drug-pricing pressure could be a multi-year overhang, not a one-year one. The CMS "Most-Favored-Nation" pricing initiative spooked pharma marketing budgets; 16 of 17 named customers were in MFN negotiations. If lower realized drug prices structurally shrink promotional budgets, OptimizeRx's whole TAM compresses.

Moat erosion. This is the most credible long-term bear point. PulsePoint (WebMD/Internet Brands) reportedly multiplied its EHR reach ~10x; Doceree claims 150+ EHR integrations including Epic and Cerner; DeepIntent (which just partnered with OptimizeRx) raised \$637m and is expanding. OptimizeRx is increasingly reselling its inventory to rivals rather than being the exclusive gatekeeper. If authenticated EHR inventory becomes a commodity, pricing power and the SSP thesis both weaken.

Conclusion

Thesis is mostly driven by valuation. I think 5x EBITDA is too cheap for this asset. Risk-reward is favorable.

I do not hold a position with the issuer such as employment, directorship, or consultancy.

I and/or others I advise hold a material investment in the issuer's securities.

Catalyst

Revenue stabilizing as the MFN air pocket laps. Reaffirming \$21m EBITDA for the year in August.

Messages

Subject Use of capital
Entry 06/25/2026 04:52 AM
Member Nick Carraway

Thanks for the write-up, how confident are you that they can hit the revised guide? It's very back half weighted. Additionally, what is a reasonable assumption on SBC? This has run high historically and is a real expense given the stock price, probably should be removed from ebitda. Now with the debt refinancing, what's the use of cash?

Subject Re: Use of capital
Entry 06/25/2026 11:03 AM
Member templargin

Hi Nick, thank you for comments.

1. "Fairly confident" would be the answer. I think market prices that as "likely to cut more". But even with a slight cut on EBITDA valuation is not demanding (even though market would react negatively likely).
2. SBC improved over the last 3-4 years as an absolute number and as % of revenue. I'd not expect more than \$8m in SBC this year. Good point though, the "true" ebitda multiples are higher than in writeup. Noted.
3. I think they will keep paying down the debt aggressively.

Subject Re: Re: Use of capital
Entry 06/25/2026 12:04 PM
Member Nick Carraway

Thanks, why would they not buy stock given gross debt/ebitda is below 1x and net-debt/ebitda is minimal? They're diluting shareholders by 10% here with the SBC

Subject Re: Re: Re: Use of capital
Entry 06/25/2026 12:13 PM
Member templargin

Good question. They announced buyback provision but when I spoke to management they said they are not in "opportunistic trading" and will do BB to keep a floor. With Whetstone ownership and their person on the board I was bit surprised by that answer. Definitely something to push on.

Subject Q2 2026
Entry 08/14/2026 10:03 AM
Member templargin

Company reaffirmed the guidance and stock popped.

They did \$4m of FCF in H1 2026 (removing \$4m of SBC). I think they'll do \$10m FCF in 2026. With EV today of \$185m OPRX is trading at 18.5x FCF. This seems like a fair valuation but it does not imply any growth. Company has a crazy operating leverage that they already demonstrated in 2025. Except opex is lower, gross margins are now in 70s and debt, which is lower now, was refinanced.

Two things to critic: no buybacks even below \$5 - they have 10b5-1 instead of opportunistic buys and SBC is creeping up again.

Any signs of returning to growth will re-rate the stock even higher.